CURRENT EDITION · APRIL 2026

THE ALETHEIA VOICE PACK

Heard. Understood. Me.

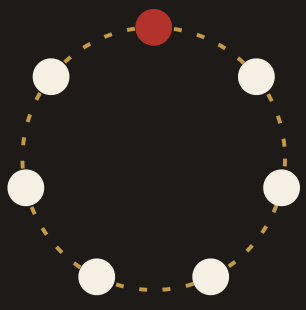
The Aletheia Project's guide to health, self-advocacy and medical dismissal for young people.

Your voice is part of your health. This pack brings together everything Aletheia has learned about helping young people understand what's happening in their bodies, say it clearly, find their way through healthcare, hold their ground, and change things for the next person.

01 UNDERSTAND · 02 ARTICULATE · 03 NAVIGATE · 04 ADVOCATE · 05 CONNECT · 06 ACT · 07 INFLUENCE

About 50 minutes, or jump to what you need · Revised September 2026 For ages 12–25

Heard. Understood. Me.



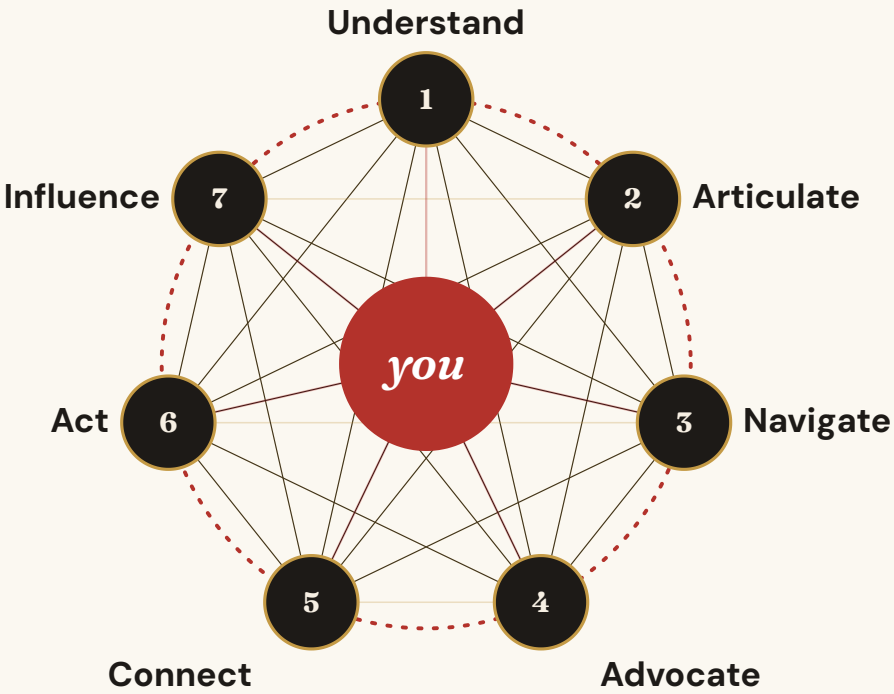
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START HERE

01 One pack, seven things you can do

Earlier Voice Packs came in stages: first knowing your body, then finding the words, then speaking up, then changing things. Young people taught us that real life doesn't come in stages. You might need the words and the courage and the right service all in the same afternoon.

So this edition is built as one connected model, the Aletheia loop. Every tool in it is tagged with the parts of the loop it helps with, and you can start wherever you are.



1 Understand
Know what's happening and the words for it.

2 Articulate
Turn what you feel into a clear symptom story.

3 Navigate
Know where to go, what it costs, what to bring.

4 Advocate
Hold your ground when you're not heard.

5 Connect

6 Act

Find people who get it, and who can

7 **Influence**
Add your voice to change what others face.

Take the next step: follow-up, feedback, referral.

Start where you are

UNDERSTAND

Something feels off

Find the words for what's happening.

ARTICULATE

I have an appointment

Build your symptom story and plan.

ADVOCATE

I wasn't heard

What to say, and what to do next.

INFLUENCE

I want to change things

Turn experience into change.

i **What this pack is, and isn't**

It's health education, written by young people and checked against Australian health sources (listed at the end). It isn't medical advice, and it can't diagnose you. It can help you get the most out of the people who can. In an emergency, call 000.

HEALTH 101 · UNDERSTAND

02 Language is power

There are moments when something feels wrong in your body, but you don't know how to explain it. You say "I just feel off", and the conversation moves on without you. Not because your experience isn't real, but because it hasn't been named.

This section gives you those names. Not so you can diagnose yourself, but so you can recognise patterns, ask better questions and stand your ground. You don't need to become an expert in medicine. You only need enough language to be heard.

Your body and your cycle

Period pain

medical word: dysmenorrhoea

WHAT IT IS

Cramping pain in the lower tummy, back or thighs around a period, caused by the uterus contracting. Common, especially in the teens.

WHAT IT CAN FEEL LIKE

Aching or cramping waves, sometimes with nausea, diarrhoea or headaches.

WORTH CHECKING WHEN

It stops you going to school or doing things, the usual pain relief doesn't help, it's getting worse each year, or you have pain outside your period.

YOU CAN SAY

> "My period pain stops me going to school, and the usual pain relief doesn't help."

> "It's been getting worse every year."

UNDERSTAND ARTICULATE

Heavy menstrual bleeding

WHAT IT IS

Bleeding heavy enough to affect your physical, social or emotional life. It's common, and it can be treated.

WHAT IT CAN FEEL LIKE

Changing a pad or tampon every 1–2 hours, bleeding through at night, clots bigger than a 50-cent coin, periods over 7 days, feeling tired or dizzy.

WORTH CHECKING WHEN

Any of the above. Heavy periods are a common cause of low iron. Urgent: soaking a pad every hour for several hours, or feeling faint.

YOU CAN SAY

> "I'm changing a pad every hour or two on my heavy days."

> "Could we check my iron levels?"

UNDERSTAND ARTICULATE

Endometriosis

en-doh-mee-tree-OH-sis

WHAT IT IS

Tissue similar to the lining of the uterus grows outside it, causing inflammation and pain. About 1 in 7 people assigned female at birth in Australia are diagnosed by their late 40s. It can start in the teens.

WHAT IT CAN FEEL LIKE

Severe period pain, pain between periods, pain with bowel movements or weeing, pain during sex, fatigue.

WHY IT GETS MISSED

Symptoms are often normalised as ‘just bad periods’. Australia’s national guideline puts the average delay to diagnosis at 6 to 8 years.

WORTH CHECKING WHEN

Period pain stops your life, or pain is getting worse over time. A GP can refer you to a gynaecologist. Diagnosis may involve an ultrasound, MRI or keyhole surgery.

YOU CAN SAY

> “My pain affects my daily life. Could this be something like endometriosis?

> “What would it take to investigate that?

UNDERSTAND ARTICULATE ADVOCATE

PMOS (formerly PCOS)

polyendocrine metabolic ovarian syndrome

WHAT IT IS

A common hormonal and metabolic condition affecting about 8–13% of women of reproductive age. In May 2026 it was renamed from polycystic ovary syndrome, because it affects the whole body and many people with it don’t have cysts. You’ll still hear “PCOS” for a while.

WHAT IT CAN FEEL LIKE

Irregular or missing periods, acne, extra hair on the face or body, hair thinning on the head, changes in weight or blood sugar.

GOOD TO KNOW

In teenagers, diagnosis needs both irregular cycles and signs of higher androgen (‘male’) hormones. An ultrasound isn’t used to diagnose PCOS until 8 years after periods start.

YOU CAN SAY

> “My cycle has been irregular for a long time, and I’ve had acne and extra hair growth too.

> “Could this be PMOS (PCOS)? What tests would help?

UNDERSTAND ARTICULATE

Hormonal changes

WHAT IT IS

Hormones regulate your cycle, mood, energy, skin and sleep. They shift naturally through puberty and across each cycle.

WORTH CHECKING WHEN

Ongoing symptoms that disrupt your life: very irregular cycles, severe mood changes before periods, persistent acne, unexplained tiredness.

GOOD TO KNOW

'Hormonal imbalance' is a phrase you'll hear a lot, but it isn't a diagnosis. If someone says it, it's fair to ask which hormone, and what test would show it.

YOU CAN SAY

> "Which hormone do you think is involved?"

> "Is there a test that would show that?"

UNDERSTAND ARTICULATE

Iron deficiency

WHAT IT IS

Not having enough iron. It's common in young people with periods, especially heavy ones, and can lead to anaemia (low red blood cells).

WHAT IT CAN FEEL LIKE

Tiredness that doesn't lift, being short of breath, dizziness, pale skin, poor concentration, feeling cold.

WORTH CHECKING WHEN

You're always exhausted, especially if your periods are heavy. It's checked with a simple blood test.

YOU CAN SAY

> "I'm exhausted all the time and my periods are heavy. Could we check my iron?"

UNDERSTAND ARTICULATE

Mind, brain and energy

Anxiety

WHAT IT IS

Your body's alarm system. Useful in small doses. It becomes a problem when it's frequent, intense, or out of proportion to what's happening.

WHAT IT CAN FEEL LIKE

Constant worry, racing thoughts, trouble sleeping, a fast heartbeat, nausea, tight muscles, avoiding things.

WORTH CHECKING WHEN

It's most days, for weeks, or it stops you doing things. headspace (12–25) and your GP are good first stops.

YOU CAN SAY

> "I've felt anxious most days for a while, not just occasionally."

> "It's affecting school and sleep. What support is there?"

UNDERSTAND ARTICULATE CONNECT

Depression

WHAT IT IS

More than sadness. A health condition affecting how you think, feel and function over weeks or longer. It isn't a personal failure.

WHAT IT CAN FEEL LIKE

Low, flat or numb, losing interest in things, changes in sleep or appetite, low energy, feeling worthless or hopeless.

WORTH CHECKING WHEN

It's lasted two weeks or more. If you have thoughts of suicide or self-harm, get help now: call Lifeline on 13 11 14, Kids Helpline on 1800 55 1800, or 000 in an emergency.

YOU CAN SAY

> "I haven't felt like myself for a while."

> "I think I need support with my mental health."

UNDERSTAND ARTICULATE CONNECT

ADHD

WHAT IT IS

A neurodevelopmental difference in how the brain regulates attention, activity and impulses. In girls it's often the less visible, inattentive type.

WHAT IT CAN FEEL LIKE

Losing focus, forgetting things, starting lots and finishing little, time slipping, restlessness inside, exhaustion from masking.

WHY IT GETS MISSED

It can be mistaken for being disorganised, dreamy, anxious or 'not trying'. Many girls are diagnosed later than boys.

WORTH CHECKING WHEN

It's been going on since childhood and affects school, work or relationships. A GP can refer you for assessment.

YOU CAN SAY

> "I've struggled with focus and organisation for as long as I can remember.

> "Could I be assessed for ADHD?

UNDERSTAND ARTICULATE ADVOCATE

Sensory overload

WHAT IT IS

When your brain gets more input (noise, light, touch, crowds) than it can process at once. Common for autistic and ADHD people, and for anyone who's unwell, exhausted or stressed.

WHAT IT CAN FEEL LIKE

Suddenly overwhelmed, irritable or panicky. Needing to leave, cover your ears or go quiet.

AT APPOINTMENTS

You can ask for adjustments at appointments: a quieter room, dimmer lights, fewer people, written information.

YOU CAN SAY

> "I'm feeling overwhelmed by the room. Could we dim the lights or find somewhere quieter?

ARTICULATE ADVOCATE

Fatigue

WHAT IT IS

Persistent exhaustion that doesn't go away with rest. Different from normal tiredness after a busy day.

WHAT IT CAN FEEL LIKE

Waking up drained, brain fog, simple tasks taking huge effort.

WORTH CHECKING WHEN

It's lasted weeks, it's affecting your life, or it comes with other symptoms. It has many possible causes (iron, thyroid, mental health, sleep, infections), so it deserves a proper look.

YOU CAN SAY

> "This isn't normal tiredness. I'm exhausted even after resting.

> "Can we look for possible causes?

UNDERSTAND ARTICULATE

When it's long-term

Chronic pain

WHAT IT IS

Pain lasting more than three months. Over time the nervous system can become more sensitive, so pain can continue even when tissues have healed. It is real pain.

WHAT IT CAN FEEL LIKE

Constant or recurring pain that affects sleep, mood, concentration and movement.

WHY IT GETS MISSED

When nothing shows up on a scan, people are sometimes told it's 'in their head'. Pain doesn't have to be visible to be valid.

YOU CAN SAY

> "This pain has been ongoing for months and it affects my daily life.

> "Can we talk about a plan to manage it, not just the cause?

UNDERSTAND ARTICULATE ADVOCATE

Chronic condition

WHAT IT IS

A health condition that lasts a long time and needs ongoing management, like asthma, endometriosis, diabetes or depression.

GOOD TO KNOW

A GP can create a care plan that coordinates your care, and may give access to Medicare-rebated allied health visits.

YOU CAN SAY

> "This seems to be long-term. Can we make a plan for managing it?

UNDERSTAND NAVIGATE

Inflammation

WHAT IT IS

The immune system's response to injury or infection. Short-term, it helps you heal. Long-term inflammation is part of some conditions, like endometriosis, inflammatory bowel disease and arthritis.

GOOD TO KNOW

'Inflammation' on its own is a process, not a diagnosis. Some blood tests show it, but not all inflammation shows up in them. The useful question is what might be causing it.

YOU CAN SAY

> "Could there be an inflammatory cause for this?

> "What tests would help show that?

UNDERSTAND ARTICULATE

Words for the healthcare itself

Diagnosis

WHAT IT IS

Working out what's causing your symptoms, based on your story, an examination and sometimes tests. It can take several appointments.

GOOD TO KNOW

A doctor may give a 'working diagnosis' while they rule other things out. It's fair to ask what else is being considered.

YOU CAN SAY

> "What could this be?"

> "What else are you considering, and how will we rule it out?"

UNDERSTAND NAVIGATE

Informed consent

WHAT IT IS

Agreeing to a test or treatment after understanding what it is, why it's suggested, the risks and the alternatives. You can ask questions, take time, say no or change your mind.

IF YOU'RE UNDER 18

Under 18, you can usually consent yourself if the doctor thinks you understand the decision (a 'mature minor'). The exact rules vary by state.

YOU CAN SAY

> "Can you explain what this involves?"

> "What are my options, including waiting?"

> "I'd like some time to think about it."

NAVIGATE ADVOCATE

Confidentiality

WHAT IT IS

Health professionals must keep your information private, except in limited cases, such as a serious risk of harm.

IF YOU'RE YOUNG

From 14, Medicare generally needs your consent before releasing your claims information to parents. From 15 you can have your own Medicare card.

YOU CAN SAY

> "Will this stay confidential?"

> "Is there anything you'd have to share, and with whom?"

NAVIGATE ADVOCATE

Implicit bias

WHAT IT IS

Assumptions people hold without realising, which shape whose pain is believed and what gets investigated. In Victoria's Inquiry into Women's Pain, one in three women said they'd felt dismissed or unheard by practitioners.

WHY IT MATTERS

Bias isn't always intentional, but it has real effects. Recognising it can help you see that being dismissed isn't about you.

YOU CAN SAY

> "I don't feel my concerns have been fully understood yet."

> "Can we take another look at my symptoms?"

UNDERSTAND ADVOCATE INFLUENCE

Referral

WHAT IT IS

A letter from your GP to a specialist or service. For most specialists you need one to get a Medicare rebate.

ASK ABOUT

Ask why, who, how much it costs, how long the wait is, and how long the referral lasts. Public clinics may cost less but have longer waits.

YOU CAN SAY

> "Would a specialist help here?"

> "Is there a public option, or someone closer?"

NAVIGATE ACT

Second opinion

WHAT IT IS

Seeing another health professional about the same issue. It's a normal part of healthcare and doesn't need anyone's permission.

GOOD TO KNOW

You can ask for copies of your test results to take with you.

YOU CAN SAY

> "I'd like a second opinion."

> "Could I have a copy of my results?"

ADVOCATE ACT

Self-advocacy

WHAT IT IS

Speaking up for your needs and your health. It isn't about being loud or perfectly confident. It can be one question, one pause, one "I don't understand".

YOU CAN SAY

> "This is important to me.

> "I'd like to understand this better.

ADVOCATE

ARTICULATE

03 Your symptom story

Doctors think in patterns: what, where, when, how much. Turn what you're feeling into six short lines and you can say it in thirty seconds, even when you're nervous.

My symptom story

6 lines · 30 seconds

WHAT	*Sharp, cramping pain* What it feels like, in your words
WHERE	*Low tummy, left side, into my back* Use the body map numbers
WHEN	*First 2 days of every period, for 18 months* Start, pattern, how long
HOW MUCH	*Usually 6, worst 9. Missed 5 school days this term* Your number AND what it stops
SO FAR	*Ibuprofen helps a bit. Heat pack. Not enough* What you've tried
I WANT	*To find out why, and a plan that works* What you need from today

Write yours on your phone or in the Self-Advocacy Journal and read it out. Reading from notes is completely normal.

What & where

Use feeling words: cramping, sharp, burning, throbbing, dragging, pressure. Then the exact place, and whether it spreads.

> "Sharp, low on the left, into my back."

When & how much

When it started, when it happens, how long it lasts. Give your usual number and your worst number, plus what it stops you doing.

> "Usually 5, worst 8. I miss the first day of school."

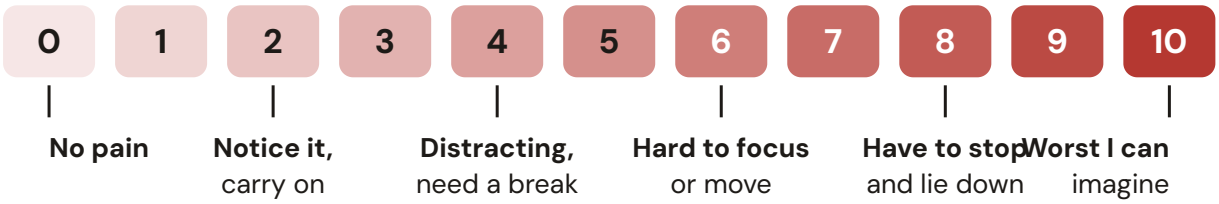
So far & I want

What you've tried, and what you need today: an explanation, a test, a plan, a referral, or to be believed.

> “ I'd like to find out what's causing it.

Scoring pain by what it stops you doing

Rate what the pain stops you doing, not just how it feels.



Give your usual number AND your worst: “Usually 4, at worst 8.”

Track it for two or three cycles

A few weeks of notes turns “it's bad sometimes” into a pattern anyone can see. Track bleeding, pain, mood, energy and missed days. The **Self-Advocacy Journal** has a tracker built in.

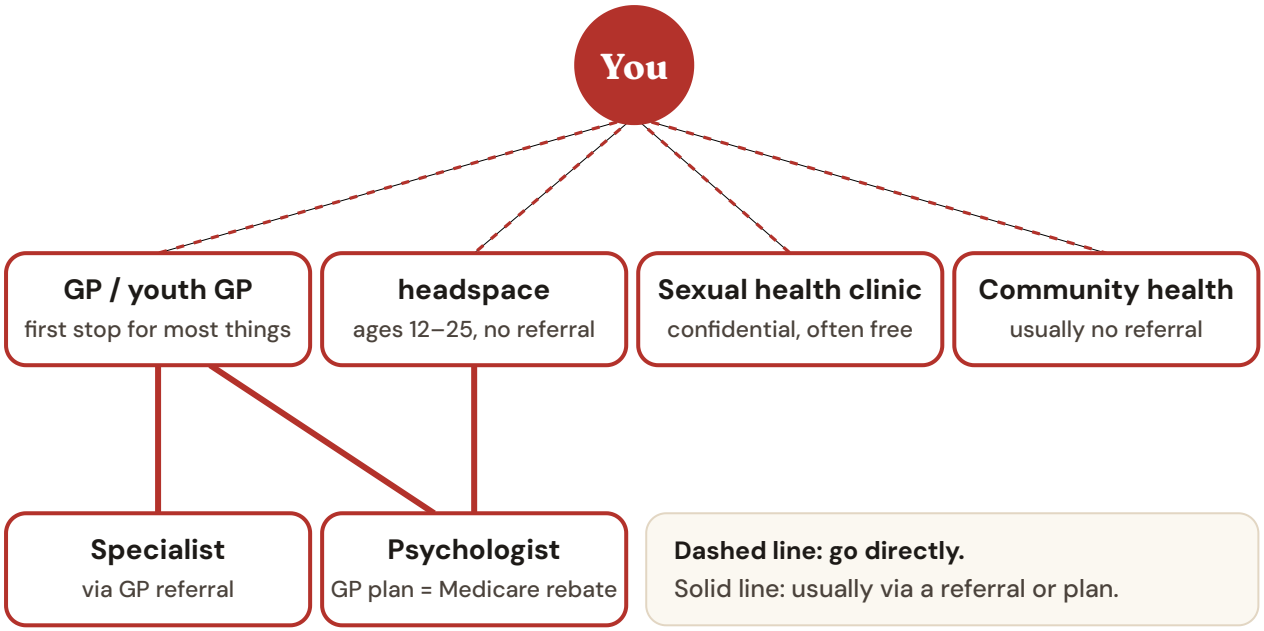
NAVIGATE

04 Before, during, and after the appointment

Healthcare can feel confusing, not because it's impossible to understand, but because nobody teaches you how it works. Knowing where to go, what it costs and what to bring changes the whole experience.



Where to go



QUICK OR URGENT HELP, STRAIGHT FROM YOU

Pharmacist	Urgent Care Clinic	Hospital ED / 000
advice, no appointment	walk in, not life-threatening	serious or emergency

For the full map, costs and what to bring, see the **Aletheia Health Navigator**.

QUESTION	THE SHORT ANSWER
Will it cost anything?	Bulk-billed GPs charge Medicare directly, so you pay nothing. Private billing means paying up front and claiming part back. Ask when you book: "Do you bulk bill young people?"
Do I need a Medicare card?	It helps. From 15 you can get your own. Many sexual health clinics and headspace centres can see you without one.
How do I get rebated psychology?	Your usual GP can make a Mental Health Treatment Plan, which gives Medicare rebates for up to 10 individual sessions a year. headspace doesn't need a referral.
What if I need an interpreter?	Ask when booking. GPs can book one for free through TIS National.
Can I do it by phone or video?	Yes. Telehealth works well for follow-ups, results and mental health appointments.

Booking

- “I’d like a long appointment. I have a few things to discuss.
- “Do you bulk bill?
- “Could I see a female GP?

Before you leave

- “What’s the plan?
- “What should make me come back sooner?
- “Could you note this in my file?

ADVOCATE

05 When you’re not heard

You might walk out of an appointment knowing something wasn’t right. Maybe you were interrupted, rushed, or told it was “just stress” before anything was explored. That feeling matters.

Medical dismissal isn’t always obvious. It can sound polite. It can come from people who mean well. This section isn’t about confrontation. It’s about clarity: for each moment, what might be going on, what you can say, and the next move. Say it quietly or firmly, however feels natural. Advocacy doesn’t look one way.

IF YOU HEAR

“It’s probably just stress.”

WHAT MIGHT BE GOING ON

Stress does affect the body. But it’s sometimes offered before physical causes are checked.

YOU CAN SAY

“I understand stress can play a role. I’d still like to rule out physical causes.”

“What would you want to check before putting this down to stress?”

NEXT MOVE

Ask what would change their mind, and when to come back if it doesn’t improve.

IF YOU HEAR

“That’s normal.”

WHAT MIGHT BE GOING ON

Something can be common and still not be manageable for you.

YOU CAN SAY

“I know it may be common, but it isn’t manageable for me.”

“At what point would this need investigating?”

NEXT MOVE

Describe what it stops you doing. Impact is harder to dismiss than a number.

IF YOU HEAR

“You’re overreacting.”

WHAT MIGHT BE GOING ON

How calm you seem says nothing about what you’re feeling.

YOU CAN SAY

“It feels significant to me, and it’s affecting my life.”

“I’d like to focus on the cause rather than my reaction.”

NEXT MOVE

Ask for your concern to be noted in your file.

IF YOU HEAR

“You’re too young for that.”

WHAT MIGHT BE GOING ON

Many conditions, including endometriosis, PCOS, ADHD and depression, can start in the teenage years.

YOU CAN SAY

“I understand I’m young, but this is affecting me now.”

“Can we look into it properly, regardless of my age?”

NEXT MOVE

Bring your tracker: a pattern over months is hard to wave away.

IF YOU HEAR

“It shouldn’t hurt that much.”

WHAT MIGHT BE GOING ON

Pain is personal. It doesn’t have to match anyone’s expectations to be real.

YOU CAN SAY

“I’m not asking you to agree with the level of pain. I’m asking for help understanding it.”

“It’s consistent, and it’s getting worse over time.”

NEXT MOVE

Use your symptom story, and ask what else it could be.

IF YOU HEAR

“Okay, so we’ll just—” (you feel rushed)

WHAT MIGHT BE GOING ON

Appointments are short. Important details get lost when things move too fast.

YOU CAN SAY

“I don’t feel I’ve explained everything yet.”

“Could we book another appointment to go through this properly?”

NEXT MOVE

Next time, book a long appointment and lead with your main concern.

IF YOU HEAR

(You're interrupted mid-sentence.)

WHAT MIGHT BE GOING ON

Interruptions are common in busy clinics, and can cut off key information.

YOU CAN SAY

"I'd like to finish explaining this before we move on."

"There's one more part that I think is important."

NEXT MOVE

Keep your notes in front of you, and go back to where you stopped.

IF YOU HEAR

"Let's just wait and see." (you're not taken seriously)

WHAT MIGHT BE GOING ON

Waiting can be reasonable, but only with a plan.

YOU CAN SAY

"This has been affecting me consistently. It's important to me that we look into it."

"What are we watching for, and when do I come back?"

NEXT MOVE

If you still feel unheard, ask for a second opinion.

IF YOU HEAR

(Medical words you don't understand.)

WHAT MIGHT BE GOING ON

Clinicians use shorthand. It's their job to make sure you understand.

YOU CAN SAY

"Could you explain that in simpler terms?"

"So, just to check, you're saying... Is that right?"

NEXT MOVE

Ask for it in writing, or for something you can read at home.

IF YOU HEAR

(You feel intimidated.)

WHAT MIGHT BE GOING ON

There's a power imbalance in the room. You don't have to perform confidence.

YOU CAN SAY

"I'd like to ask a few questions before we continue."

"I need a moment to process this."

NEXT MOVE

Bring a support person, or ask to pause and come back.

IF YOU HEAR

“So, shall we go ahead?” (you’re not ready)

WHAT MIGHT BE GOING ON

Most decisions aren’t urgent. You’re allowed time.

YOU CAN SAY

“I’d like some time to think before deciding.”

“Is there any risk in waiting a little?”

NEXT MOVE

Take the information home, look it up on a trusted site, and come back with questions.

IF YOU HEAR

(The appointment isn’t in your language.)

WHAT MIGHT BE GOING ON

You have a right to understand your care. GPs can book a professional interpreter for free.

YOU CAN SAY

“I would like an interpreter for this appointment.”

“Can we speak without my family translating?”

NEXT MOVE

Ask when you book, so the interpreter is ready. TIS National is 131 450.

IF YOU HEAR

(The way it’s explained doesn’t work for you.)

WHAT MIGHT BE GOING ON

If you process information differently, the standard format may not fit.

YOU CAN SAY

“I may need more time to process this.”

“Could you explain it step by step, and write it down?”

NEXT MOVE

Ask for reasonable adjustments when you book.

IF YOU HEAR

(You want it on the record.)

WHAT MIGHT BE GOING ON

Notes create a timeline, and make the next appointment faster.

YOU CAN SAY

“Could you please include that in my medical record?”

“I’d like this concern to be noted.”

NEXT MOVE

Keep your own record too: date, what you said, what they said.

IF YOU HEAR

“I don’t think we need tests.”

WHAT MIGHT BE GOING ON

Sometimes true. But you can ask for the reasoning.

YOU CAN SAY

“Is there anything that would help us understand this better?”

“I’d feel more comfortable ruling out possible causes first.”

NEXT MOVE

Ask what would make testing appropriate, and note the answer.

IF YOU HEAR

“Come back if it gets worse.”

WHAT MIGHT BE GOING ON

Without a clear plan, it’s easy to fall through the gaps.

YOU CAN SAY

“What should I do if this doesn’t improve?”

“How long should I wait before coming back?”

NEXT MOVE

Book the follow-up before you leave the clinic.

IF YOU HEAR

(You still don’t feel it’s resolved.)

WHAT MIGHT BE GOING ON

A second opinion is a normal and responsible part of care.

YOU CAN SAY

“I’d feel more comfortable getting a second opinion.”

“Could you refer me to another specialist?”

NEXT MOVE

Take copies of your results and your symptom story.

> **Being taken seriously isn’t something you earn. It’s something you’re entitled to.**

NAVIGATE · ADVOCATE

06 Your rights in the Australian system

The Australian Charter of Healthcare Rights applies to everyone getting care in Australia, whatever their age. Knowing it turns “I hope they’ll listen” into “I’m entitled to be involved”.

RIGHT	WHAT IT MEANS FOR YOU
Access	Healthcare that meets your needs, including interpreters and adjustments.
Safety	Care that’s safe, in a place where you feel safe.
Respect	Being treated with dignity, with your culture, identity, beliefs and choices respected.
Partnership	Asking questions and being involved in decisions, with support people if you want them.
Information	Clear information about your condition, options, risks and costs, in a way you understand.
Privacy	Your personal and health information kept private and confidential.
Give feedback	Commenting on or complaining about your care, and having it taken seriously.

Consent

You can ask what a test or treatment involves, why, the risks and alternatives. You can take time, say no, or change your mind. Under 18, you can usually consent yourself if you understand the decision; the rules vary by state.

Confidentiality

What you say is private, with limited exceptions such as serious risk of harm. From 14, Medicare generally needs your consent to share claims with parents; from 15 you can have your own card. Sexual health clinics are confidential and often free.

If something went wrong

1. **Raise it with the clinician or practice**
   If it feels safe. Ask for the practice manager if you prefer.
2. **Put it in writing**
   What happened, when, and what you’d like done. Keep a copy.

3

Go to your state's health complaints body

It's free. In Victoria, that's the Health Complaints Commissioner. Complaints are also how services find patterns and change.

CONNECT

07 Care that fits who you are

Culture, language, gender, sexuality, disability and age shape how people are heard. Good care adapts to you. Here's what you can ask for.

Culturally and linguistically diverse

Ask for a professional interpreter, free at GP appointments. It's okay to see a doctor without family present. If health is taboo at home, confidential services exist for exactly this.

> "Can we speak without my family translating?

LGBTQIA+

You choose what to share. You can correct your name and pronouns, and ask if a service is inclusive. QLife (1800 184 527) offers peer support.

> "I use ___ pronouns.

Aboriginal and Torres Strait Islander

Aboriginal Community Controlled Health Organisations offer culturally safe care, run by community. 13YARN (13 92 76) is 24/7 crisis support.

Neurodivergent or disabled

Ask for adjustments: more time, a quiet room, written information, step-by-step explanations, a support person.

> "Could you write that down for me?

Young

Your age is a reason to be thorough, not to stop looking. You can see a GP on your own, and ask to speak alone for part of an appointment.

> "Could I have some time with the doctor on my own?

Faith and family

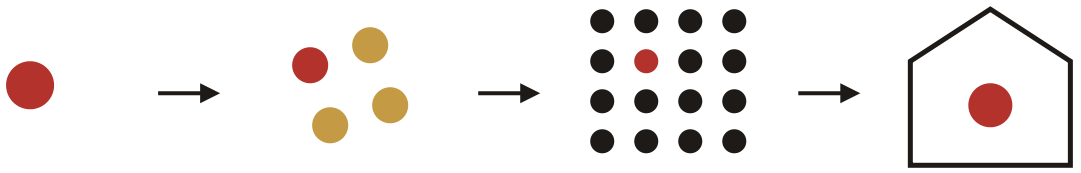
You can ask for care that respects your beliefs, like a doctor of a particular gender, or modesty during an examination.

> "Is it possible to see a female doctor?

ACT · INFLUENCE

08 From me to we

Every time a young person is dismissed, it affects them. When it happens to thousands, it tells us something about the system. Aletheia exists to connect the two: to help you get care today, and to turn what young people go through into evidence that changes what the next person faces.



Me
my experience

Us
others have it too

Evidence
a pattern we can show

Change
a decision shifts

THE EVIDENCE

13,000+

responses to Victoria's 2024 Inquiry into Women's Pain, where one in three women said they'd felt dismissed or unheard.

THE EVIDENCE

6–8 years

average delay to an endometriosis diagnosis in Australia, with symptom normalisation named as a key cause.

THE RESPONSE

Youth voice

Young people's experiences, gathered safely and shared with decision-makers, are how that pattern starts to shift.

Sharing your story safely

- [ ] I'm sharing because I want to
- [ ] I know where it will go and who will see it
- [ ] I've chosen my name, first name only, or anonymous
- [ ] I've left out details that identify other people
- [ ] I can review or withdraw it before publishing
- [ ] I have someone to talk to afterwards

Where your voice goes at Aletheia

These are the structures we've built so young people's experience reaches the people who make decisions:

- **Youth Consultation**: your input on real decisions
- **Youth Roundtables**: young people and decision-makers in one room
- **Youth Research** and the **Youth Research Journal**: lived experience as evidence
- **Aletheia Voices** and Aletheia Talks: sharing stories and writing
- **Youth Executive Council**: young people leading Aletheia

You're not just reading this. You can help write the next one.

Every Voice Pack is shaped by young people. Join the resource development or translation teams, or tell us what should be in the next edition. **See all the ways to get involved**.

REFLECT

09 Your story, your voice

There are so many places where you're expected to have answers. This isn't one of them. Your experience doesn't need medical language, or to sound "serious enough". It only needs to be yours.

Write here, think here, or leave it blank. The moment you start putting your experience into words, you're no longer carrying it alone.

PAGE 1

What have you been told about your body?

1. What have you been told is normal?
2. What have you been told isn't worth mentioning?
3. What has your body been telling you, despite that?

PAGE 2

What do you wish someone believed?

1. Something physical: a pain that never fully went away?
2. Something emotional: a weight you carried quietly?
3. Something you're still trying to understand?

PAGE 3

What does your body feel like today?

1. Is there tension, heaviness, lightness, discomfort anywhere?
2. Is there something you've been ignoring?
3. What would you tell a doctor about today?

PAGE 4

What does advocating for yourself look like?

1. What feels manageable right now: one question? Writing things down?
2. Who could back you up?
3. What would you say if your voice shook?

PAGE 5 · NEW IN THIS EDITION

What would you change for the next person?

Your experience can make it easier for someone else. What would you want a doctor, a school or a government to understand?

1. One thing I wish had been different...
2. Who could change it...
3. One small step I could take...

SUPPORT

10 When to get help, and where

Most things in this pack are about everyday care. Some things can't wait. Know the difference.

EVERYDAY

Keep an eye on it

- New symptoms that are mild
- Things you want to understand better
- Track it, and see a GP if it keeps up

BOOK A GP

Worth an appointment

- Symptoms stopping school, work or plans
- Pain or bleeding getting worse
- Low mood or anxiety most days for two weeks or more
- Anything that worries you. That's reason enough.

URGENT · 000

Get help now

- Sudden, severe pain
- Very heavy bleeding with dizziness or fainting
- Trouble breathing, chest pain
- Thoughts of suicide or harming yourself, or someone else is in danger

Emergency

000

If you or someone else is in danger or needs urgent medical help.

Kids Helpline

1800 55 1800

Free, 24/7 phone and webchat, ages 5–25.

Lifeline

13 11 14

24/7 crisis support. Text 0477 13 11 14.

headspace

headspace.org.au

Free or low-cost support, ages 12–25.

13YARN

13 92 76

24/7 crisis support by Aboriginal and Torres Strait Islander people.

QLife

1800 184 527

LGBTIQ+ peer support, 3pm–midnight.

healthdirect

1800 022 222

24/7 nurse advice: is this urgent, and where should I go?

TIS National

131 450

Free interpreting, when you need to talk to a service in your language.

Beyond Blue

1300 22 4636

24/7 mental health support.

1800RESPECT

1800 737 732

24/7 support for sexual, domestic and family violence.

Butterfly Foundation

1800 33 4673

Support for eating disorders and body image.

Suicide Call Back Service

1300 659 467

24/7 counselling for anyone affected by suicide.

THE ALETHEIA PROJECT

11 What Aletheia is, and why we exist

The Aletheia Project is a youth-led not-for-profit. It exists because of a gap many young people feel but can't always name: the gap between having access to healthcare and actually knowing how to use it.

Aletheia is Ancient Greek for truth. We're not here to replace healthcare. We're here to strengthen your position in it, and to make sure young people's truth reaches the places where decisions get made.

HEALTH LITERACY

Understanding your body, and the language for it.

SELF-ADVOCACY

Being able to speak, ask and hold your ground.

SYSTEMS CHANGE

Young people shaping the services and policies that affect them.

CONTACT

Get in touch

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Want printed copies for your school or organisation? Email us. Want to see how we got here? **Explore the Voice Pack Archive**.

> **Your body is yours. You're allowed to question it, understand it, and speak about it. And when you do, you already begin to change it.**
>
> The Aletheia Project, 2026

Health education, not medical advice. This pack explains general health information so you can understand your body and talk about it. It cannot diagnose you. If you're worried about your health, speak to a doctor, nurse or other health professional. **In an emergency, call 000.**

Emergency · 000
If you or someone else is in danger or needs urgent medical help.

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Free, 24/7 phone and webchat, ages 5–25.

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▶ **Sources and fact-checking (20)**

Written, designed and fact-checked by The Aletheia Project's youth resource development team, building on four earlier editions and the young people who shaped them. Scenarios are fictional. We acknowledge the Wurundjeri Woi-wurrung people of the Kulin Nation, where Aletheia was founded. Sovereignty was never ceded.